

Cutaneous Manifestations in SLE

The high prevalence of certain cutaneous features in systemic lupus erythematosus (SLE) has led to their inclusion—past or present—in the American College of Rheumatology (ACR) criteria for diagnosing SLE.

Photosensitivity

Photosensitivity, defined as an abnormal cutaneous reaction to ultraviolet radiation (UVR), is a relatively sensitive indicator of SLE. Studies have reported incidence rates between 50% and 93% among SLE patients. Given its frequency, all patients with SLE should be educated on the importance of protecting their skin from UVR exposure.

Oral Ulcers

Oral ulcers occur in approximately 25% to 45% of individuals with SLE and up to 25% of those with discoid lupus erythematosus (DLE). While biopsy may reveal lupus erythematosus (LE)-specific histopathologic changes—particularly in DLE—these lesions are often histologically non-specific.

Alopecia

LE-non-specific alopecia refers to diffuse, non-scarring hair loss that differs from the scarring alopecia seen in DLE. Often termed "lupus hair," it presents as coarse, dry, and fragile hair that breaks easily, sometimes more pronounced along the frontal hairline. This form of alopecia likely overlaps significantly with diffuse non-scarring alopecia caused by telogen effluvium.

Cutaneous Vascular Reactions

Vascular involvement in the skin is common in SLE and may reflect underlying systemic vascular pathology.

Raynaud Phenomenon

Raynaud phenomenon is the most frequent vascular reaction in lupus, affecting up to 39.6% of SLE patients. Its presence is associated with higher disease activity and a worse prognosis. In SLE patients, Raynaud phenomenon may also correlate with an increased occurrence of DLE lesions on the hands.

Chronic, severe Raynaud phenomenon can lead to several cutaneous changes, including: - Focal ulcerations on fingertips and periungual areas, leading to pitted scarring - Nail fold capillary ectasia and dropout - Punctate cuticular hemorrhages due to capillary incompetence - Fingertip tuft atrophy - Digital calcinosis - Pterygium inversus unguis

Vasculitis

Patients with SLE may develop vasculitis affecting vessels of all sizes. When leukocytoclastic vasculitis (LCV) is observed, secondary causes such as medications or infections must be ruled out before attributing it to lupus.

Urticarial vasculitis is not uncommon in SLE and typically presents as tender urticarial papules and plaques—often over bony prominences—that persist longer than 24 hours.

Involvement of medium or large vessels may manifest as: - Purpuric plaques with stellate or retiform borders - Cutaneous necrosis or ulceration - Subcutaneous nodules

It is important to differentiate LE-non-specific vasculitis from vasculopathy due to thromboembolism, as clinical presentations can overlap. LE-non-specific vasculopathy is frequently associated with antiphospholipid antibodies.

Livedo Reticularis

Livedo reticularis is considered a distinct LE-non-specific cutaneous vascular reaction, though it overlaps with vasculopathy. It appears as a net-like, blanchable pattern of red-purple rings, typically on the extremities, resulting from impaired blood flow through dermal vessels.

Other Vascular Findings

- **Macular erythema** over the hypothenar and hyperthenar eminences of the palms has been reported in SLE.
- **Reticulated palmar erythema** may indicate vasculopathy linked to antiphospholipid antibodies.
- **Nail fold capillary dilation** is observed in LE patients but is less common than in dermatomyositis (including clinically amyopathic forms) or systemic sclerosis.